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Data Management Plan

Version 1.0

Dated September 16th, 2020

KISS

Keep bIfurcation Single stenting Simple

KISS STUDY

Study Sponsor: CERIC, 3 rue des Granges, 1204 Genève, Switzerland

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1. Scope

The "Data Management Plan" (DMP) is a document comprising multiple sections that describe all aspects of data management activities for a specific study

The "Data Management Plan" is a living document initiated after validation of the Clinical Investigation Plan (Appendix 1) and may be updated throughout the project lifecycle until the database lock, according to data management activities list of requirements (Appendix 2).

2. List of abbreviations and definition of terms

Term	Definition
CDM	Clinical Data Manager
CEC	Clinical Events Committee
CIP	Clinical Investigation Plan
COM	Clinical Operations Manager
CPL	Clinical Project Leader
CRA	Clinical Research Associate
CRF	Case Report Form
CTA	Clinical Trial Assistant
DM	Data Manager
DMP	Data Management Plan
DVP	Data Validation Plan
EDC	Electronic Data Capture
eTMF	Electronic Trial Master File
MD	Medical Director
POT	Proximal Optimization Technique
SB	Side Branch
SIV	Site Initiation Visit
SOP	Standard Operating Procedure
UAT	User Acceptance Test(ing)

3. Study description

The KISS study is an international, multicenter, randomized, prospective, post-market study designed to evaluate the non-inferiority of no side branch (SB) intervention versus side branch ballooning, in the setting of single stenting with systematic proximal optimization technique (POT), in the treatment of bifurcation lesion.

The KISS study is designed to randomly allocate a total of 596 subjects in a 1:1 ratio (test: control) to achieve a minimum of 596 evaluable subjects. All subjects will undergo clinical follow-up assessments at 1 and 12 months.

For more details, please refer to the applicable version of the Clinical Investigation Plan (CIP) (Appendix 1)

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4. Procedures to be used for data management activities

Please refer to the applicable version of the Quality Plan of the KISS study.

5. Roles involved in data management activities

Role	Main responsibilities
Sponsor Representatives	Review and validation of the following documents: • Data Management Plan • Data Validation Plan • the Case report Form and specifications • List of requirements for Data management activities
Clinical Operations Manager CERC	Review and validation of the following documents: • Data Management Plan • Data Validation Plan • Case report Form and specifications
Clinical Project Leader CERC	Draft of the Case Report Form Review and validation of the following documents: • Data Management Plan • Data Validation Plan • List of requirements for Data management activities • User Acceptance Test Coordination of the User Acceptance Testing Providing the list of manual checks to DM
Data Manager CERC	Preparation of Data Management Monthly Reports Preparation and validation of the following documents: • Data Management Plan • Data Validation Plan • Data Management Report • List of requirements for Data management activities • EDC modification according to User Acceptance Test Review of the Case report form and specifications Design and update(s) of the EDC EDC administration Database lock at the end of the study.

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6. Data Management Steps [*EDC*]

6.1. Computerized systems used for data management activities

Computerized System	Descriptive documentation	Validation Status	Provider's Adress
Castor	Electronic Data Capture https://helpdesk.castoredc.com/	Designed to meet the system requirement of 21 CFR part 11 (FDA) and EU-GDPR requirements	Castor EDC George Westinghousestraat 2, 1097 BA, Amsterdam The Netherlands
SAS v9.4	Software developed for data management and advanced analytics	Qualified	SAS France Domaine de Grégy Grégy-sur-Yerres 77257 Brie Comte Robert Cedex

6.2. CRF design

The Clinical Project Leader (CPL) collects all necessary documents and information to review the Case Report Form (CRF), then organizes a meeting with the Data Manager (DM), the Medical Director (MD), the Clinical Operation Manager (COM) and the Sponsor to validate a first version.

The CPL sends the draft CRF for review to the sponsor and the coordinator investigator and implements the appropriate comments.

Once the sponsor and the Clinical Project Leader (CPL) expectation on the CRF are met, the CPL sends to the Data Manager the CRF validation form (CT_SOP_002a)

Once the CRF is validated, the DM prepares the Data Validation Plan (DVP) (cf Appendix 3) that should include appropriate rules and checks.

The DVP contains the following information

- Document history section
- Database structure and advanced logic manual checks

Once finalized and validated, each subsequent modification of the CRF is collected in the CRF history table (DM-SOP-002b).

6.3. Design of Electronic Data Capture System (EDC)

6.3.1. EDC System

Castor is the system used as Electronic Data Capture, which can be accessed via the following internet address: https://data.castoredc.com/

See Appendix 4 for the user guide for this system.

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6.3.2. Database structure specifications

The DM designs the EDC according to the CRF and specifications validated by CERC and the Sponsor.

6.3.3. Patient Identification Code

The patient identification will be in the numerical format [Country-code]-[Site ID]-[001] incremented by one.

6.3.4. User Acceptance Testing (UAT)

Prior to data entry, an UAT (at least two rounds) will be performed by the CPL, the COM and at least one of the Clinical Research Associate (CRA) involved in the study.

All findings resulting from the UAT will be recorded on CERC standard EDC system findings document (DM_SOP_009b) including:

- Identification of the findings
- Initials of the tester
- Version of the CRF used
- Number of the EDC release
- Date of UAT
- Type of observation (request if not indicated in the CRF or in the DVP, findings if indicated and not programmed)
- CRF page where the observation is detected
- Description of the observation
- Status of the observation

The document is sent to the DM to be resolved, after performed modification(s) the system will be re-tested according to the above-mentioned process, until the EDC is compliant with the required specifications.

6.3.5. User profiles design

Please refer to the table in Appendix 5.

6.3.6. EDC and Access management

6.3.6.1. EDC deployment

EDC deployment means that a new EDC release is accessible for study users.

The first deployment happens when the study goes live, at the beginning of the study. Other deployment can happen when a request is made in order to modify the EDC.

Once all findings or requests of the UAT are resolved, the DM prepares the "EDC Deployment Authorization Form (Appendix 8) (DM_SOP_009c) and sends it for validation and signature to:

- the Sponsor,
- the CPL,

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- The DM

Once the EDC Deployment Authorization form is signed by the CPL, the DM and the sponsor, the EDC can be deployed.

In case of any modification during the course of the study, the sponsor will need to sign the EDC Deployment Authorization only if he participated in the UATs, or in case of a major impact of the EDC leading to changes in the database structure. In case of minor Impact on the EDC without changes in the database structure (i.e. wording, number format, check, rule), the Sponsor's signature is not required.

6.3.6.2. Access management

For any user involved in the KISS study, the CPL or CRA fills in the "EDC System Grant and Remove Access Request" (Appendix 9) (DM_SOP_009a) for requesting the creation, deactivation or modification of a user access to the EDC.

The DM :

- centralizes and authorizes the requests for account creation, deactivation or modification
- creates, deactivates or modifies a user account (login, password) and documents it on the "EDC System Grant and Remove Access Request"

Once the account is created, the password is generated automatically by the system, which also sends automatically an email to the user with the login and a temporary password. This password is requested to be changed at the first connexion of the respective user.

Project team

Once the CPL, CRAs, COM, Clinical Trial Assistant (CTA), CEC chairperson, Core lab Specialist and Core lab Manager users have been trained to the study protocol and to the EDC, the CPL sends to by email at the address for EDC account the "EDC System Grant and Remove Access Request" form completed with the user name, the site name and number, the user profile and the email address of the user.

When creating the account, the DM completes the data management part of the form, signs it and stores it in the Data Management File of the electronic Trial Master File (eTMF). The DM informs the requestor by email that the account is created.

The login and a temporary link to create a password are generated automatically by the system and sent to the email address of the user.

Site Staff

During the Site Initiation Visits (SIVs), CRAs will collect for each person attending the SIV (i.e. trained to the study and to the EDC) and who will be delegated by the Principal Investigator for performing data entry/signature activity, the authorization to use her/his professional contact details to create login and password to access the study EDC. This will be accomplished by completing, signing and dating the "EDC Site Access Request Form" (CT-SOP-013i). Based on this document the CRA sends the completed EDC System Grant and Remove Access Request Form to the DM. If a user site retires from the study, the CRAs or CPL sends a new form to the DM to request deactivation of the account.

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If a user retires from the study, the DM completes, signs and stores the “EDC System Grant and Remove Access Request Form” with removal access proof in the Data Management File of the eTMF.

Every 6 months the DM send to the CPL the full list of user accounts. The CPL check against the current list of approved users to verify that only current users have active accounts.

6.4. Data Validation

6.4.1. Data Validation Plan

The DVP created for the KISS Study contains all automatic data consistency checks used in the EDC.It contains also the list of checks not made directly on EDC named Manual checks. The list of manual checks is provided by the CPL and reviewed by MD, COM and DM.

In case an EDC update becomes necessary or an update of the manual checks during the course of the study, the DVP will be updated accordingly.

Please refer to the DVP of the KISS Study for the full list of electronic checks.

6.4.2. Data Consistency checks

6.4.2.1. Automatic Data Clarification

Queries may be raised automatically by the EDC system at any time during the study.

The queries are to be answered by Site staff (Principal Investigator, Investigators or Study coordinator). The queries are closed manually by CRA or DM.

The query may have different status:

- New
- Open: The user has acknowledged/opened the query, and added a remark. The status changes from New to Open.
- Unconfirmed: The user does not agree with the monitor.
- Confirmed: The user agrees with the monitor and will try to resolve the issue.
- Resolved: The user has changed the value and indicates the issue is resolved
- Closed: The monitor indicates the issue is resolved and marks the query as closed. The query turn from red to green.

6.4.2.2. Manual Data Clarification (meaning queries posted)

Queries may be raised by CRAs at any time during the study during data verification. The workflow is identical once posted to automatic queries (please refer to 6.4.2.1 section).

Queries that cannot be automatically raised in the EDC can be manually raised by the DM by programming checks in SAS on the dataset exported directly from the EDC.

The list of manual queries is written in the DVP in the spreadsheet “Manual queries”.

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The DM sends the listing to the CERC CPL who is in charge to distribute the queries to CRAs, in order to correct the corresponding data under the EDC.

6.4.3. Data deletion

If a form is completed by mistake in the EDC:
- The site requests the CRA by email to delete the form,
- or the CRA requests the site to delete the form through a manual query

The CRA completes the “Deletion Request Form” (Appendix 10) (DM_SOP_009e) and transmits it to the DM, with the email from the site requesting the deletion attached. The DM checks the requests deletes the form.

6.4.4. Database quality control

The quality control consists in ensuring that all data reported on the EDC by the site are presents in the database exported from the EDC. The export is done on .xml format and imported on SAS by the DM.

The first quality control is performed when 5 patients are enrolled.

Further controls are performed at each major database structure modification or before any snapshot / lock.

DM selects randomly 5 patients and prints the respective CRF of these patients. The DM then verifies that each data present on the paper CRF is imported on the SAS database.100% of this data will be controlled.

The DM documents in the "Data Management Report" (Appendix 7) a summary of the database quality controls performed all along the course of the Clinical Study).

6.4.5. Database reconciliation

- Not applicable

6.4.6. Database lock and snapshot

“Data Base lock” means the action to remove writing access from the clinical study database.

“Data base snapshot” means the action to export data and lock until a specific timepoint, in order to performed an interim analysis.

A snapshot is planned planned at the end of inclusion (meaning all patients have performed the post-procedure visit, which corresponds to the primary endpoint)? and a lock at the end of the study

Before a lock, all queries and checks activated on the EDC should be closed, and all forms should be signed.

The DM fills in the "Database Lock Request Form" (Appendix 11) (DM_SOP_007a) to document
- The database status at the time of the Lock request, summarizing

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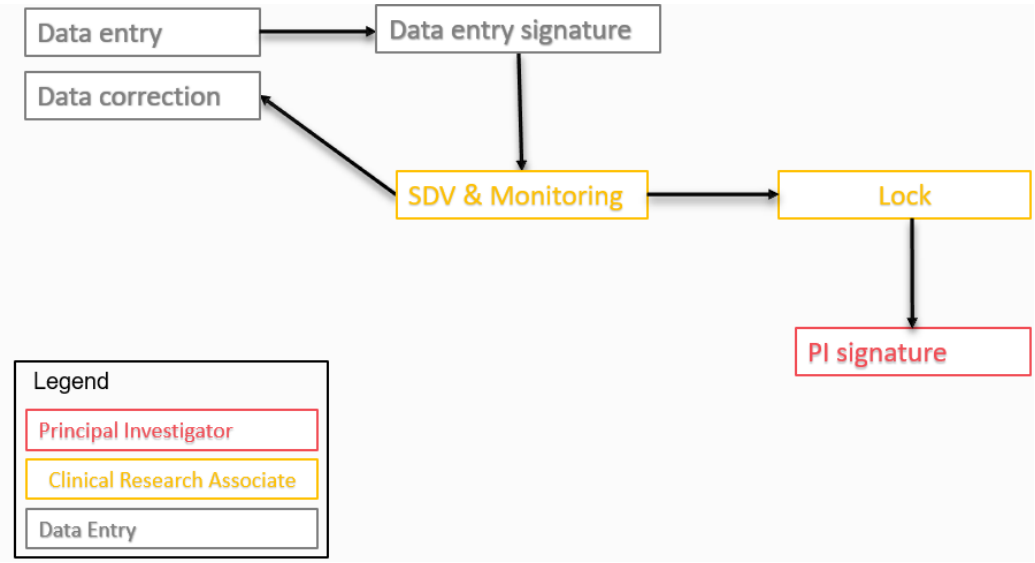
- if applicable, handling of Case Report Forms,
- handling of the derived variables,
- possible missing data,
- handling of the queries,
- handling of coding activities,
- handling of external data,
- performance of quality controls and audits,
- any other relevant verification regarding the data management activities planned for the clinical trial.

The DM dates and signs the Database Lock Request Form and transmits it for authorization to the CPL who transmits it to the sponsor.

On receipt of the Database Lock Request Form, the CPL and the Sponsor
- Review the information related to the database status, prepared by the Data Manager,
- Date and sign the Database Lock Request Form, authorizing the database lock.

6.4.7. Workflow



6.5. Clinical Events Committee (CEC) and Corelab data

CEC adjudication results are entered directly into the EDC by the chairperson. The entered information is only visible for the CEC chairperson, the DM, the COM and the CPL. Therefore, they are not visible for sites and sponsor.

6.6. Management of External Data

An external database is used for Corelab analysis. The corelab database is periodically send to the CDM for reconciliation with EDC data and completion status according to the imaging performed by the patient and entered into the EDC.

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6.7. Annotated CRF

When creating the database structure, the DM associates to each question of the CRF a variable name, label and format.

The annotated CRF (cf Appendix 6) is downloaded from the Castor in pdf format.

The annotation is automatically performed by the EDC system.

For each form, next to each field, a specific number is noted, indicating the field unique reference ID, label, format, conditional actions and methods of calculation, which have been previously defined by DM.

3. Baseline - Demographics

Number	Question	Answers
3.1	Gender *Field type*: Radiobutton *Variable name*: sex *Field required*: Required *Option group name*: sex	◯ Female ◯ Male
3.2	Year of birth *Warning shown if field's value is larger than 2002: 'Year of birth is prior to 2002. Please check'* *Field type*: Year *Variable name*: birth_year *Field required*: Required *Field min*: 1900.00 *Field max*: 2020.00	[ ] *(yyyy)*
3.3	Age *Warning shown if field's value is smaller than 18: 'Should be up to 18'* *Field type*: Calculation *Variable name*: *Field required*: Not required *Measurement Unit*: Year	*Template:* var birthyear = moment('{birth_year} 'YYYY'); var visitdate = moment('{consent_dt}', 'DD-MM-YYYY'); visitdate.diff(birthyear, 'y');

6.8. Monitoring

According to the Monitoring Plan of the KISS study

- 100% of data will be source data verified in 10% of patients at each site (with a minimum of 2 patients per site)
- 100% of patients will be source data verified from Inclusion to 12-month FU for selected data
- 100% of endpoints-related events will be source data verified

The 10% CRFs (patients) to be 100% monitored at each site will be randomly determined by using an excel file prepared by the DM. For this purpose, the CRA will perform the randomization that will be approved by the CPL before the visit.

The CPL and CRAs are trained by the DM to the system functionalities related to monitoring. This includes queries management (send a query, answer a query, close a query or reactivate a query).

Data can be marked as verified by the CRA during source data verification, Once the appropriate EDC pages have been completed without errors (no open queries) and then *lock* meaning the data is on read only mode. If needed, CRA can also unlock using the same process.

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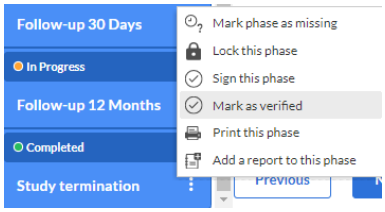
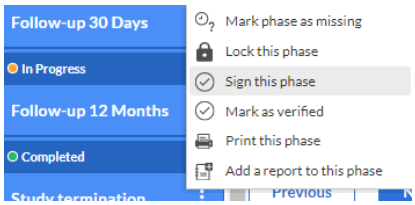
The *Mark as verified, Lock this phase* and *Unlock this phase* Monitor button, to be used for monitoring by the CRA, appear at the left of the record as highlighted below:



Once the page is verified by the CRAs, the page is ready for data entry signature.
The *Sign this phase/steps* button, to be used by the data entry to sign the page, appear at the left of the record as highlighted below:



If the site makes a modification on a verified or signed data, the CRA will have to marked as verified and the data entry will have to sign again.

To validate all data entered for a patient, the Principal Investigator Signature section must be completed only by the PI.

6.9. Self-evident corrections

Self-evident correction can be made by the DM if the correction cannot be made directly in the EDC. The list of self-evident correction should be validated first by the investigator and then the CPL.

A request has to be made by the CPL to the DM specifying the patient, the visit, the form, the field, the value and the correction. The DM programs these modification in the SAS import program of the data base.

The list of self-evident correction is available in the final Data Management Report issued at the end of the study.

6.10. Data coding

Not Applicable.

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6.11. Periodic data management reporting

Every month, the DM sends the DM status to MD and CPL to have an overview of the study indicators about completion, follow-up tracking and any indicator that can be useful for the CPL or MD.

6.12. Data Blind Review

The Blind Data Review Meeting (BDRM) will take place after all data have been collected and cleaned and prior to the database lock.

The CDM coordinates the meeting by issuing the appropriate line listings, the patients' profiles and the listing of all the protocol deviations.

The Committee takes decisions on the following subjects:

- Classification of the protocol deviations (e.g. minor, major),
- Patient classification for study outcome (e.g.: success, failure),
- Patient classification for statistical analysis (e.g.: population "Intent to Treat","Per Protocol".

The CPL issues the minutes of the meeting and attached in appendix the Analysis Sets Specification Document where all decisions will be fully documented

6.13. Data Management Report

The DM reports all actions not mentioned in the DMP and that may have an impact on the statistical analysis.

This document is completed during the course of the study and is finalized at the end of the study.

The Data Manager Team Leader, the CPL and the sponsor validate the report at the end of the study.

6.14. EDC Training

Each user is trained on the use of the EDC according to his/her profile. The DM trains the project management team (COM, CPL, CRA, CTA (if applicable) and specific users involved in the study (Core lab Specialist/reviewer, CEC member (chairperson)).

All training records are kept in the eTMF.

During the SIVs, the CRA (or the CPL) trains the site staff on the use of the EDC. If the project management team considers that a retraining is deemed necessary (for example in case of recurrent problems), the CRA, or the CPL contacts the site to organize a new training. Each site training has to be documented on the "Site staff training form" (CT_SOP_013m).

All EDC users will be provided with electronic copy of the EDC user guide. Each time that updates are required to the EDC user guide, the DM will perform the required update. The CPL will send the updated user guide for review to CERIC before distribution to the EDC users.

6.15. Database transfer

In compliance with the applicable data protection regulation (GDPR (EU) 2016/679), before sending the database to an external biostatistician, the DM ensures that the database is protected by a password.

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The transfer is done via two different channels:

- Using an internal secured access platform to upload the database,
- Using an email containing the passwords to access the database which is sent by the Data Manager to the receiver.

6.16. Data anonymisation

As no data anonymisation is expected in the scope of this study, this section is not applicable.

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7. Appendices and related documents

- Appendix #1: Clinical Investigation Plan
- Appendix #2: Data management activities list of requirements
- Appendix #3: Data Validation Plan
- Appendix #4: EDC User Guide
- Appendix #5: User profile design
- Appendix #6: Annotated Case Report Form
- Appendix #7: Data Management Report template
- Appendix #8: EDC Deployment Authorization Form
- Appendix #9: Grant and remove access request form
- Appendix #10 Deletion Request form
- Appendix #11: Database Snapshot / Lock request form
- Appendix #12: Database Snapshot /Lock Certificate

Certificat de réalisation

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État: Complétée

Objet: Veuillez signer avec DocuSign : KISS DMP v1.0

Enveloppe source:

Nombre de pages du document: 16

Signatures: 5

Nombre de pages du certificat: 5

Paraphe: 0

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État: Original
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Accepté: 19/09/2020 08:44:57
ID: e2ac1309-dd72-46bd-89a4-39bb12e1fac8

Événements de signataire	Signature	Horodatage
Valery ELIE velie@cerc-europe.org Clinical Project Leader CERC Niveau de sécurité: E-mail, Authentification de compte (aucune)	DocuSigned by: Valery ELIE 06958D7DB44D48B... Sélection d'une signature : Style présélectionné En utilisant l'adresse IP: 86.247.181.86	Envoyée: 18/09/2020 08:57:24 Consultée: 18/09/2020 08:57:39 Signée: 18/09/2020 08:57:40

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Événements de livraison à l'éditeur	État	Horodatage

Événements de livraison à l'agent	État	Horodatage

Événements de livraison intermédiaire	État	Horodatage

Événements de livraison certifiée	État	Horodatage

Événements de copie carbone	État	Horodatage

Événements de témoins	Signature	Horodatage

Événements notariaux	Signature	Horodatage

Récapitulatif des événements de l'enveloppe	État	Horodatages
Enveloppe envoyée	Haché/crypté	18/09/2020 08:57:24
Remise certifiée	Sécurité vérifiée	19/09/2020 08:44:57
Signature complétée	Sécurité vérifiée	19/09/2020 08:45:43
Complétée	Sécurité vérifiée	19/09/2020 08:45:43

Événements de paiement	État	Horodatages

Divulgation relative aux Signatures et aux Dossiers électroniques

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